

PRIMARY LOGIC

Compact Business Plan

2026-08-13 | 2.1 - TOTAL OWNED OUTCOMES

Commercial rule: Primary Logic is paid for every verified kept first appointment it produces from an assigned referral. Baselines prove customer value; they do not reduce the invoice.

1. Company and offer

Primary Logic is the administrative operating system for specialty referral execution. The EHR remains the system of record; Primary Logic becomes the system of action. It owns each permitted administrative next step across the patient, payer, clinic, schedule, and EHR until every assigned referral reaches one known terminal state.

Assign Primary Logic an eligible referral lane. We own every administrative step from receipt through a kept first appointment or another defined terminal state. You pay a fixed fee for every kept patient we produce - not for seats, hours, attempts, or statistically estimated lift.

Decided

- The customer transfers an eligible referral lane, not isolated tasks.
- Every qualifying kept first appointment from that lane is billable.
- Declined, unreachable, disqualified, escalated, customer-blocked, and no-show outcomes are not billed in the initial product; their cost is priced into successful outcomes.
- Historical baselines and holdouts evaluate value, not invoices.
- Clinical judgment remains with the customer; the EHR governs assignment, status, and billing evidence.

Unvalidated

- Dermatology as the first specialty.
- A \$75 to \$150 fee per kept first appointment, with \$100 as the starting hypothesis.
- Kept rates, delivery costs, sales cycles, gross margins, and removable customer operating cost.
- The exact federal and state legal structure for per-kept-patient compensation.

2. Product boundary

Assignment

A referral becomes Primary Logic's responsibility when the customer records an eligible referral and assignment timestamp in the agreed system of record. It must have stable identifiers, no qualifying appointment, lawful contact authority, sufficient routing context, and a market with available or expected capacity. Customer staff stop parallel outreach except through the escalation path; exclusive ownership prevents duplicate work and creates clean attribution.

Primary Logic owns

- Ingestion, normalization, deduplication, eligibility, and next-action selection.
- Approved voice, SMS, email, and form workflows; identity, consent, and contact-policy state.
- Demographic, insurance, intake, document, eligibility, and administrative payer work.
- Capacity-aware scheduling, confirmation, reminders, cancellation recovery, and rescheduling through the first visit.
- EHR read/writeback; attempt, decision, dependency, ownership, and audit history.
- Context-rich human or clinical escalation and terminal-state reconciliation.

Every assigned referral ends as: **kept first appointment; declined; unreachable; administratively ineligible; human or clinical escalation; or customer-caused block.** Only the kept first appointment is initially billable.

Primary Logic never diagnoses, triages, prescribes, selects treatment, overrides clinical rules, invents documentation, makes unsupported payer attestations, guarantees coverage, or continues contact after revocation.

Product invariant

Every referral has one current state, one accountable owner, one next action or terminal state, one decision history, and one synchronized EHR record. Patient communication is an execution channel inside this control plane.

3. Wedge, customer, and expansion

Land and expand

Land with one visibly failing lane: unscheduled referrals, an aged queue, one region, one specialty, one EHR workflow, and sufficient capacity. After safe ownership is proven, expand upstream to every eligible referral from receipt. Then add separately priced outcomes such as cancellation recovery, intake completion, authorization resolution, referral closure, recall scheduling, or medication-access administration. Each module requires its own objective invoice event.

Beachhead customer

A centralized multi-site specialty group should have:

- 25,000 to 40,000 eligible annual referrals in initial or near-term scope.
- A credible path to 8,000 to 10,000 annual kept outcomes for a seven-figure contract near \$100 per outcome.
- Central patient-access ownership, one dominant EHR or data layer, reliable event timestamps, rules-based appointment inventory, and capacity.
- Existing labor, BPO, software, overtime, or hiring spend that Primary Logic can remove, redeploy, or prevent.
- Repeatable administrative workflows with bounded clinical escalation.

Prefer PE-backed or centralized dermatology, GI, ophthalmology, ENT, orthopedics, cardiology, or comparable ambulatory platforms. The first customer should optimize for executive control, standardization, and deployment speed rather than theoretical volume.

annual contract value

= assigned referrals x kept rate x outcome fee

30,000 x 35% x \$100 = \$1.05M

60,000 x 40% x \$100 = \$2.40M

Disqualify or reprice

- No executive will transfer exclusive workflow ownership, or staff will continue parallel outreach.
- Capacity, assignment, appointment, or kept-status evidence is unreliable.
- The customer routes only its hardest inventory or cannot commit enough volume.
- Most cases require clinical judgment before administrative work can begin.
- Existing conversion is high, cost to serve is low, and no labor, vendor, software, or hiring cost can move.
- Incremental contribution plus operating leverage does not comfortably exceed Primary Logic fees.
- Contact, consent, recording, BAA, security, integration, or EHR access cannot be made safe and repeatable.

A customer need not permit a holdout. Incrementality is not the invoice model.

4. Buyer and sale

Stakeholder	Purchase case	Blocking objection
COO / platform president	Same-store growth, operating leverage, standard execution	Paying for patients who might have converted anyway
VP Patient Access / Operations	One owner and next action for every referral	Loss of control or poor escalation
CFO	Forecastable unit economics and auditable invoices	Total fee exceeds growth plus cost leverage
CIO / CISO	EHR integrity, bounded permissions, auditability	Writeback, maintenance, PHI, and vendor risk
Compliance / clinical leader	Explicit administrative boundary and human gates	Compensation, communications, and clinical risk

The COO, president, or PE operating partner is the economic buyer; the VP of Patient Access or Operations is the champion. Engage technical, security, compliance, privacy, and clinical approvers early. Avoid innovation sponsors without authority over workflow, capacity, and production volume.

Founder-led sale

1. Map the complete referral lane, ownership, queues, systems, handoffs, outcomes, and failure modes.
2. Quantify annual inventory, kept rates, capacity, operating cost, planned hiring, and contribution per incremental patient.
3. Identify which labor, BPO, software, management work, or future hiring can move.
4. Define assignment, exclusivity, clinical boundaries, terminal states, customer blocks, price, volume, and expansion before integration.
5. Launch one paid production lane; expand to all eligible referrals, then adjacent outcome modules.

Core objection: "Some patients would have converted without you."

Answer: Primary Logic is not selling an attribution claim. It takes over work the customer currently staffs, buys, manages, and audits. Compare the total fee with incremental patient contribution plus operating cost removed, labor redeployed, software replaced, and hiring avoided. If that value does not clear the fee, the account is not qualified.

5. Pricing and contract

invoice = all eligible kept first appointments x contracted outcome fee

- No seat, call, message, minute, form, attempt, or incrementality charge.
- Initial hypothesis: \$75 to \$150 per kept first appointment; target starting point: \$100.
- Price varies with committed volume, referral difficulty, kept rate, channel cost, integration, and escalation burden.
- Require a complete lane, minimum eligible volume, or annual outcome envelope to prevent under-routing and adverse selection.
- Standard implementation may be free, credited, or recovered through the minimum; bespoke work is separately priced or refused.

Billable outcome

A billable converted patient is an eligible referral assigned exclusively to Primary Logic with no qualifying appointment at assignment, for whom Primary Logic creates or materially causes a qualifying first appointment that the authoritative EHR records as kept inside the attribution window.

The contract must define eligibility, assignment, exclusivity, qualifying appointment, attribution window, self-scheduling after contact, customer intervention, duplicates, cancellations, reschedules, no-shows, late arrivals, completed visits, capacity failures, reconciliation, dispute windows, and audit rights.

Historical baselines may prove value; a limited holdout may establish causality when operationally acceptable. Neither reduces production invoices. Primary Logic is paid for completed work, not only the estimated difference from a counterfactual.

6. Customer and company economics

Keep revenue, value, and counterfactual impact separate.

Primary Logic revenue

= total eligible kept outcomes x outcome fee

customer net value

= incremental kept outcomes x contribution per incremental patient

+ labor, BPO, software, management, and hiring cost removed or avoided

- Primary Logic fee

- customer implementation and change cost

Structural illustration

Input	Illustration	Status
Assigned annual referrals	60,000	Assumed
Historical / Primary Logic kept rate	35% / 40%	Assumed
Total / incremental kept appointments	24,000 / 3,000	Derived
Contribution per incremental patient	\$650	Assumed
Incremental contribution	\$1.95M	Derived
Outcome fee / Primary Logic annual fee	\$100 / \$2.40M	Assumed / derived
Customer implementation cost	\$250K	Assumed
Operating-cost leverage required for breakeven	\$700K	Derived

This case is deliberately uncomfortable: growth alone does not justify a \$100 fee on all kept outcomes. The customer must also remove, redeploy, or avoid at least \$700,000 of annual operating cost. Never treat contribution from patients who would already have converted as value newly created by Primary Logic.

Qualification evidence

Collect referral and outcome volume by market; time to contact, book, and keep; capacity; fully loaded patient-access labor; BPO, telephony, messaging, scheduling, and point-software spend; open roles, overtime, and planned hiring; staff time by state and exception; implementation cost; and contribution per incremental first patient.

Primary Logic unit economics

Most cost occurs per assigned referral; revenue occurs per kept outcome.

delivery cost per kept outcome

= delivery cost per assigned referral / kept rate

+ allocated deployment, support, and escalation cost

gross margin

= (outcome fee - delivery cost per kept outcome) / outcome fee

At a \$100 fee, 40% kept rate, and 60% gross-margin target, total delivery cost must remain below \$40 per kept outcome. Before allocated deployment and support, that implies less than \$16 per assigned referral. Measure voice, messaging, inference, payer transactions, integration, escalation, quality, compliance, and customer support rather than assuming software margins.

7. Paid deployment and implementation

Launch one complete eligible lane - not selected hard cases - with roughly 3,000 to 10,000 referrals or enough volume for 60 to 120 days of operating evidence. Bill every qualifying kept outcome at the contracted deployment price. Agree on expansion pricing and decision gates before launch.

Success gates

- At least 95% scheduling and writeback accuracy.
- At least 95% of referrals always have an owner and next action or terminal state.
- At least 90% reach an auditable terminal state inside the service window.
- No material case-mix-adjusted increase in no-shows.

- Escalations meet service levels; no uncontained privacy, consent, or safety incident.
- Positive Primary Logic contribution margin and positive customer economics combining growth with operating leverage.
- Written expansion decision before the deployment ends.

Standard launch

- Weeks 1 to 2: BAA, security, economics, outcome definition, and workflow mapping.
- Weeks 2 to 4: integrations, communications, scheduling rules, and state reconciliation.
- Week 5: synthetic, internal, and customer acceptance testing.
- Week 6: limited production with daily review.
- Weeks 7 to 12: production ramp, weekly operating review, and monthly invoice reconciliation.
- Weeks 10 to 16: expansion decision and broader lane assignment.

Reject bespoke work unless it creates a reusable specialty, payer, scheduling, or EHR capability.

8. Safety, product, and moat

Minimum readiness includes a BAA and downstream BAAs; encryption, least privilege, logging, incident response, retention, and deletion; channel-specific consent, opt-out, revocation, and call-recording controls; administrative-only policies; human review for ambiguous identity, urgency, eligibility, attestation, or clinical content; and no PHI used for model training without explicit authorization.

Specialist counsel must review federal anti-kickback, state fee-splitting and patient-brokering, telemarketing, privacy, and recording rules. Incrementality does not remove compensation risk; the per-kept-patient model requires counsel before becoming public contract policy.

Primary references:

- [HHS sample business-associate agreement provisions](#)
- [HHS-OIG fraud and abuse guidance](#)
- [42 CFR 1001.952\(d\), personal services and outcome arrangements](#)

The required product is a control plane: complete inventory; current state, owner, dependency, and next action; attempt and decision history; exception queues; policy enforcement; capacity-aware scheduling; EHR reconciliation; invoice audit trail; and cohort economics. The execution layer adds voice, SMS, email, forms, identity, consent, payer administration, intake, scheduling, and handoff.

The moat is the operational graph accumulated across complete workflows: state transitions, action policies, payer and scheduling logic, integrations, exception handling, outcome definitions, and production performance. Do not expand specialty or outcome scope until one lane has repeatable integration, bounded escalation, stable unit economics, and reliable reconciliation.

9. Metrics and twelve-month proof

North star: billable kept first appointments produced from assigned referrals.

Lens	Required measures
Customer	Assigned referrals; booked, kept, no-show, decline, unreachable, escalation, and block rates; time to contact/book/keep; incremental contribution; operating cost moved; net value after fees
Operations	Referrals with owner and next action; terminal-state closure; attempts and cost per referral; automation and escalation; scheduling/writeback accuracy; consent, complaint, safety, and incident rates
Company	Delivery cost per referral and kept outcome; contribution and gross margin by lane; contracted volume, price, outcome envelope; launch time; expansion rate; integration hours; net revenue retention

Incremental lift is measured to prove value, not define revenue.

Milestones

Period	Evidence required
Months 0 to 3	Counsel-approved contract structure; one outcome definition; minimum security package; two paid design partners; 10,000 referrals in complete lanes
Months 4 to 6	Reliable terminal-state ownership; positive lane contribution margin; one full-lane expansion; validated fee range; launch below 45 days; one customer-approved case
Months 7 to 9	Three live customers; 150,000 annualized referrals managed; credible path to 60% gross margin; one repeatable implementation and one channel
Months 10 to 12	One \$1M annual outcome envelope; 300,000 annualized referrals contracted; two referenceable deployments; 30 to 45 day standard launch; evidence-based expansion decision

These are operating targets, not forecasts.

10. Critical unknowns

1. What fee will COOs and CFOs accept for all kept outcomes, including patients who might have converted?
2. How much labor, BPO, software, management, and hiring cost can customers actually remove or redeploy?
3. Will customers transfer exclusive ownership of a complete lane and commit enough volume to prevent adverse selection?
4. What are actual kept rates and delivery costs by specialty, payer, market, age, language, and assignment point?
5. Can per-kept-patient compensation be structured acceptably across federal-program and commercial populations?
6. Which EHR and scheduling systems concentrate fast, reliable early deployments?
7. What share of referrals can close without human or clinical escalation?
8. Does waiting for kept status create unacceptable billing delay or working-capital pressure?
9. How quickly can a recovery lane expand to every eligible referral, then adjacent outcomes?

The first customers must prove three linked facts: they will transfer a complete lane; growth plus operating leverage exceeds the fee on all kept outcomes; and Primary Logic can deliver attractive contribution margin without unsafe automation.

Canonical language: Primary Logic is the administrative operating system that owns every next step from referral receipt through a kept first appointment or another known terminal state. Assign us an eligible referral lane; we work every referral and charge a fixed fee for every kept patient we produce. Baselines prove value. Assigned kept outcomes determine revenue.